جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شيماء عيد محمد حامد
المحافظة : الجيزه **الحي / المدينة :** الاخصاص **العنوان :** ....
رقم البطاقة : 29002202100441 **رقم التليفون :** 01158780401 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 20-02-1990 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شيماء عيد محمد حامد
National ID: 29002202100441

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name:	**SHAIMAA EAD MOHAMED HAMED**
Date:	**2026-04-21**
Referred by:	Prof.Dr. CT CAP+C
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -
Known patient of metastatic breast cancer
compared to previous study dated 27/12/2025 the current study revealed

1-Chest:
Stationary course as regarding the right breast, still seen diffuse skin thickening& coarse heterogeneous parenchyma, right LOQ calific focus & prominent bilateral axillary LNs

- No sizable pulmonary nodules, masses, cavitary lesions or bronchiectasis changes.
- Right middle lobe atelectatic bands are not seen now.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2-Abdomen and pelvis:

- The liver is average in size with regressive course as regarding the focal lesions at segment II measuring 1.7x1.9cm compared to 2.3x1.9cm at the previous study and stationary course as regards the focal lesion at segment VI still measuring 1x1cm as in previous study.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3-The scanned skeleton:

- Still seen sclerosed L1 with collapse

(42)


جمهورية مصر العربية
بطاقة تحقيق الشخصية
شيماء
عيد محمد حامد
عزبة القصيبى الكبيرة
الجيزه - الجيزه
٢٩٠٠٢٢٠٢١٠٠٤٤١
١٩٩٠/٠٢/٢٠
HG5814665


٢٩٠٠٢٢٠٢١٠٠٤٤١ ٢٠٢٤/١٢
حاصله على ليسانس اداب وتربية
انثى مسلمة متزوجة
طه عبدالباسط حسن محمود مجاهد
١ البطاقة سارية حتى ٢٠٣١/١٢/٠٩

Name: شيماء عيد محمد **MR. :** 2981	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The multidisciplinary team on 15 September 2025 discussed the patient's regimen of capecitabine (XELODA) and trastuzumab deruxtecan (TYKERB). The patient received cycle 4 of capecitabine and cycle 3 of TYKERB, followed by a planned dose of TYKERB 5 mg × 1 × 30 on cycle 5 and capecitabine 3 × 2 × 14 on cycle 5. Laboratory results showed clinical improvement with hemoglobin 11.7 g/dL, total leukocyte count 6.08 ×10⁹/L, platelets 313 ×10⁹/L, urea 24.3 mg/dL, creatinine 0.55 mg/dL, ALT 18 U/L, and AST 18 U/L; a CT of the chest, abdomen, and pelvis was pending.

On 23 March 2026 the patient was started on zoledronic acid (Zometa) plus capecitabine (Xeloda) 3 + 2, and reported the onset of hand-foot syndrome. The regimen also included TYKERB 5 mg × 1 × 30.

On 28 April 2026 the case was documented as de novo metastatic breast cancer receiving TYKERB and capecitabine initiated in October 2025. The most recent CT chest-abdomen-pelvis on 21 April 2026 demonstrated stable disease of bone metastases and reduction of hepatic lesions. TYKERB was to be renewed that day, and capecitabine (3 × 2 × 14) was ordered with dispensing planned for the following Sunday, followed by a one-week rest period.

On 16 May 2026 the patient was noted to be on day 14 of capecitabine and presented for dispensing of TYKERB.

On 25 May 2026 internal medicine recorded that in May 2026 the patient had stable right breast disease with residual left-breast involvement, and the CT chest-abdomen-pelvis in April 2026 showed stable bone disease and stable liver lesions. Capecitabine 500 mg was prescribed as 3 × 2 × 14, with the next clinic visit scheduled for 15 June 2026.

On 14 June 2026 the CT chest-abdomen-pelvis from 21 April 2026 confirmed stable osseous metastases and reduction of hepatic lesions. Laboratory results were acceptable. The patient received capecitabine 3 × 3 and TYKERB 5 tablets, along with zoledronic acid on the same day; labs remained stable and the next visit was set for 4 July 2026.

On 4 July 2026 the plan was to continue capecitabine 3 × 3, with TYKERB to follow after laboratory results, and the patient was to be seen again in three weeks.

On 6 August 2026 capecitabine 500 mg was prescribed as 3 × 2 × 14, with the next TYKERB dose scheduled for 13 August 2026 and the subsequent clinic visit on 26 August 2026.

On 27 August 2026 capecitabine 500 mg was ordered as 3 × 2 × 14, with TYKERB planned for 13 September 2026 and the next follow-up appointment set for 17 September 2026.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

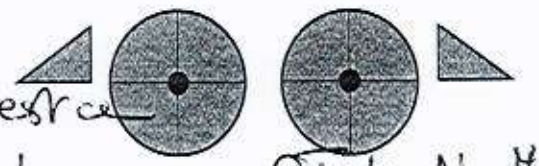
MDT FORM 15/9/2025 G

me		MRN	1991/C. CE
		ID	
e	35 Y. ♀	Phone	01158780401
enopausal status	premp	Marital status	
RS SCORE		Family history	
MI		REASON OF MDT	Remdt
morbidity			

Clinical Data

			Nr.
	PR:	HER 2:	Ki67:
-ve	-ve	3+	

R pS:1 L



cT4b, N1, M1

TNM

nomammography: premeno Rt Brest Ca locally advanced Bilateral Br. locally advanced

ets Workup: Rt dugat +L4 Met. to Bone L4 oligomet. with Newly detected Rt Left Breast axilly

Initially cT4b, N1, M1 oligomet to L4 Rt breast

commendations: Rec. of Taxotere + Herceptin → RD

Mastectomy Herceptin → po Taxotere, Carboplatin x4cy + Herceptin

→ po locally Rt'sed + Newly detected Rt Breast mets axill LN

DT OUTCOME & PLAN By CT -ve - Pt -ve, later on. cT 6/2025 → SD of L4 lesion

Pt Rec. 1st cycle of Navelbine + 2nd cycle Herceptin Refused Br pt Reported Subjective Radiological progression to Hospice

9/12

SIGNATURE

EDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
15/9/2025	15/9/2025				

2025 Pt received only 1 cycle Navelbine → Consider Xeloda/~~Kepte~~ TyKerb

JSTAVE/
OUSSY

Patient Name	: الأستاذه / شيماء عبد محمد حامد	Request No.	: 13007412
Patient No.	: 29002202100441	Request Date	: 23/07/2025
Path No.	: 2339/25	Verification Date	: 29/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data
Left breast mass, 4 o'clock lesion

Nature of specimen
Tru cut biopsy, collected on 22/7/2025.

Gross examination
Fragmented grayish white tissue cores ranging in size from 0.7 cm up to 1 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
Histological type: Invasive duct carcinoma.
Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
DCIS: Minimal <1 %, solid pattern, inetrmediate grade
Angioinvasion: Absent
Perineural invasion: Absent
IPT (Positive control showed appropriate reactivity) :
ER: No nuclear reaction in tumor cells (Allred score 0/8)
PR: No nuclear reaction in tumor cells (Allred score 0/8)
HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
Ki 67: 15 %

Diagnosis

Verified by :

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY

Patient Name	: الأستاذه / شيماء عبد محمد حامد	Request No.	: 13007412
Patient No.	: 29002202100441	Request Date	: 23/07/2025
Path No.	: 2339/25	Verification Date	: 29/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Left breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade 2 with minimal ductal carcinoma insitu , solid pattern, intermediate grade
IPT:
* ER: Negative (score 0/8).
* PR: Negative (score 0/8).
* HER2: Positive (score 3+).
* Ki67: 15 %

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Coraiche - Misr El Qadema - Cairo, Egypt

Patient ID: 86065	Patient Name English: Shaimaa Eid Mohamed Hamed
Age: ٣٥ سنة, ٥ شهر	Report Date :2025/08/11 12:40:19 PM

Women's Health Outreach Program

Contrast-Enhanced Digital Mammography

Clinical summary: Bilateral pathologically proven IDC grade II, Her-2 enriched.

Technique

MLO and CC views (low energy & recombined post contrast)

Targeted High resolution ultrasound

Detailed Findings:

Density: ACR D

Background Uptake: Mild

Right breast

- Low energy (non-contrast images) show: multiple innumerable variable sized speculated high density mass lesions with clusters of micro-calcifications and architectural distortion affecting the whole breast. Thickened skin, coarsened trabeculae and retracted nipple.
- Recombined (post contrast) images show: corresponding scattered heterogeneous enhancing masses; the largest is seen retro-areolar measuring 1.6 x 3.6 x 3.9. The intervening parenchyma showed areas of clumped non-mass enhancement.
- The pathological process involve the whole breast.
- Ultrasound demonstrated multiple malignant mass lesions scattered all over the breast with intervening altered parenchyma.
- Thickened skin reaching 7 mm and coarsened trabeculae.

CamScanner ممسوحة ضوئيا بـ

- Non-specific axillary LNs. No pathological infra or supra-clavicular lymph nodes.

Left breast

- Low energy (non-contrast images) show: UOQ, posterior third speculated irregular equal density mass lesion with surrounding distortion. No suspicious calcifications.
- Recombined (post contrast) images show: corresponding UOQ, posterior third heterogeneous mass enhancement measuring 1.2 x 1.6 cm seen 10.5 cm from nipple.
- Ultrasound demonstrated correlate 3 o'clock, posterior third malignant mass lesion.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Multiple (More than three) pathological level 1 and level 2 nodes are noted showing effaced hila and thickened cortices reaching 4 mm. No pathological supra-clavicular lymph nodes.

Impression:

Right breast (BIRADS 6): Malignant mass and non-mass pathological process involving the whole breast.

Left breast (BIRADS 6): UOQ posterior third mass enhancement with ipsilateral pathological level 1 &2 axillary lymph nodes.

Dr. Amal Hegazi, M.Sc

CamScanner